November 2026

Thomas Yu

Address: Department of Economics
Yale University
New Haven, CT 06520-8268

Telephone: +1 (469) 585-6982

Email: t.yu@yale.edu

Website: https://thomas-yu.com

Citizenship: United States

Fields of Concentration:
Health Economics, Industrial Organization

Qualifying Examinations Completed:
2023 (Oral): Industrial Organization, Econometrics

Dissertation Title: *Essays on Health Care Markets*

Committee:
Professor Jason Abaluck (Chair)
Professor Steven Berry
Professor Victoria Marone

Education:
Ph.D., Economics, Yale University, 2027 (expected)
M.Phil., Economics, Yale University, 2024
M.A., Economics, Yale University, 2023
B.A., Economics (with honors), University of Chicago, 2019
B.S., Mathematics, University of Chicago, 2019

Fellowships, Honors and Awards:
University and Teaching Fellowships, Yale, 2021–2027
Department of Economics/Cowles Foundation/EGC Fellowship, Yale, 2021–2026
Certificate of College Teaching Preparation (CIRTL Associate), Yale, 2025
David A. Gimbel Fellowship, Yale, 2025
Nathan Hale Associates Fellowship, Yale, 2024
David S. Hu Award in Economics, University of Chicago, 2019
BFI Award for Academic Achievement in Macroeconomics, University of Chicago, 2019
Phi Beta Kappa, University of Chicago, 2019
President's Scholarship, University of Chicago, 2015–2019
Dean's List, University of Chicago, 2016, 2017, 2018

Teaching Experience:
Fall 2025, Teaching Assistant to Prof. Benjamin Polak, Game Theory, Yale

Spring 2025, Teaching Assistant to Prof. Daniel Hauser, Intermediate Microeconomics, Yale
Spring 2024 and Fall 2024, Teaching Assistant to Prof. Cormac O'Dea, Introductory Microeconomics, Yale
Fall 2023, Teaching Assistant to Prof. Eva Chalioti, Intermediate Microeconomics, Yale

Research Experience:

Research Assistant to Prof. Zack Cooper, Yale, 2022–2023
Research Assistant to Profs. Tarek Hassan and Leonardo Bursztyn, University of Chicago, 2017–2019

Work Experience:

Economist, Lyft, 2019–2021
Economist Intern, Lyft, 2018

Working Papers:

"The Value of a Hospital Obstetric Unit: Access, Costs, and Health," *Job Market Paper*

"Redesigning Graduate Medical Education Payments: The Role of Residency Caps," *under review*

"Incidence of Drug Price Regulations" with Genna Liu

Publications:

"High-Frequency Location Data Show That Race Affects Citations and Fines for Speeding" (2025). Pradhi Aggarwal, Alec Brandon, Ariel Goldszmidt, Justin Holz, John List, Ian Muir, Gregory Sun, and Thomas Yu. *Science*, 387(6741), 1397–1401.

Other Research:

"Acculturation and Mental Health Help-Seeking Behavior in a Community Cohort of Individuals of Chinese Descent" (2023). Zilin Cui, Yiyang Yuan, Amy Cheung, Thomas Yu, Kun Hu, Chuyu Wang, and Xiaoduo Fan. *Asian Journal of Psychiatry*, 85, 103628.

Seminar and Conference Presentations:

2026: Allied Social Science Associations, Econometric Society North American Summer Meeting, Geography and Health: Exploring Rural-Urban Disparities (NBER Conference), American Society of Health Economists, Southeastern Health Economics Study Group

Referee Service:

American Economic Review

Professional Service:

Organizer, Yale Industrial Organization Tea, 2025

Languages:

English (native), Chinese (fluent)

November 2026

References:

Prof. Jason Abaluck
Yale University
School of Management
New Haven, CT 06520
PO Box 208200
Phone: 203-432-7811
jason.abaluck@yale.edu

Prof. Steven Berry
Yale University
Department of Economics
New Haven, CT 06520
PO Box 208268
Phone: 203-432-3556
steven.berry@yale.edu

Prof. Victoria Marone
Yale University
Department of Economics
New Haven, CT 06520
PO Box 208268
Phone: 203-432-3576
victoria.marone@yale.edu

Dissertation Abstract

The Value of a Hospital Obstetric Unit: Access, Costs, and Health [Job Market Paper]

Hospital obstetric (OB) units are closing across the United States, especially in rural areas. I study how a hospital's decision to operate an OB unit affects access to care, the costs of delivering care, and maternal and infant health in equilibrium. Then, I quantify the net externalities of OB provision and evaluate policies aimed at preventing OB unit closures.

In a simple example, an OB unit increases welfare by giving mothers access to an additional option, but it also decreases welfare due to its fixed cost of operation. Effects on variable costs and maternal and infant health are ambiguous, depending on the OB unit's marginal cost and quality relative to its competitors. A profit-maximizing OB hospital does not fully internalize its effects on mothers, infants, or other hospitals, so OB provision has a net externality that can be positive or negative.

I study these effects in Arkansas, a particularly rural state. Using data on births and medical claims from the Arkansas All-Payer Claims Database, I estimate a model of the hospital OB care market. First, hospital systems make OB provision decisions. Second, insurers and hospital systems bargain over prices. Third, mothers choose OB hospitals, and maternal and infant health is determined.

I apply the estimated model to simulate outcomes if each OB unit closed. The access-to-care effect is large, while effects on health and variable costs are small and mixed in sign. The average OB unit generates a positive net externality worth $104 per Arkansas birth. Mothers highly value having more options and shorter travel distances, but much of this value is not reflected in OB hospitals' profits.

Finally, I evaluate policies, starting with Arkansas's 2025 Healthy Moms, Healthy Babies Act, which raises the prices paid by Medicaid for OB care by 70%. The policy keeps OB units open when their fixed costs go up. Its marginal value of public funds ranges from 1.0 to 1.5: each dollar of public spending creates $0 to $0.50 in additional welfare. However, a drawback of the Medicaid price hike is that it pays more to large hospitals that would have operated OB units anyway. As an alternative, I consider a flat subsidy that pays all OB hospitals the same amount. The subsidy's marginal value of public funds ranges from 1.2 to 4.2, achieving the same goal more cost-effectively.

Redesigning Graduate Medical Education Payments: The Role of Residency Caps

Hospitals that train medical residents receive graduate medical education (GME) payments, with each hospital assigned a cap on its subsidized residents. About half of teaching hospitals exceed their caps

and thus receive no additional payments for training more residents. I study whether GME payments can be redesigned to increase the number of residents by redirecting payments from over-cap to under-cap hospitals. Instrumenting for under-cap status using changes in hospital size since the caps were assigned, I find that under-cap hospitals' resident counts respond to payments with an elasticity of 0.74 while over-cap hospitals have no response. My estimates suggest that reducing over-cap hospitals' payments by 25% and transferring the savings to under-cap hospitals would lead to 3,700 more residents without any change in government spending. These gains are proportionally larger in rural areas, which face more severe physician shortages.

Incidence of Drug Price Regulations, with Genna Liu

Prescription drug prices are determined through a chain of interactions among manufacturers, insurers, and consumers. A regulator seeking to lower drug prices must choose which point in the chain to regulate and which drugs to target. How are the effects of drug price regulations distributed among market participants, and how does that guide a regulator's preferred choices? We develop a model of a drug market with rebate bargaining and community-rated premiums. A net price ceiling transfers surplus from manufacturers to insurers, and a copay cap reaches drug users directly with insurers and manufacturers splitting the cost. Beyond that, consumers may be affected through premiums, which only move when drug users are marginal enrollees. A consumer-welfare-maximizing regulator should cap copays, while a spending-minimizing regulator can use either instrument. Copay caps should target drugs with high out-of-pocket prices, many users, and elastic utilization; net price ceilings should target high net spending. Applied to insulin in Medicare Part D, the model implies that Medicare's $35 insulin copay cap raises fills by 3% at a cost borne by insurers and manufacturers rather than by other consumers.